NCCP 00414 · clinician draft

Palbociclib with endocrine therapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Palbociclib with endocrine therapy · supportive-care review

Active medicines: Palbociclib + the prescribed endocrine partner (AI OR fulvestrant)

Emetogenic risk: Oral minimal–low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Palbociclib is classified as oral minimal–low emetic risk. Routine premedication is not usually needed; discuss rescue medication if nausea occurs. A generic intensive chemotherapy antiemetic schedule is not automatically added.
- Blood-count monitoring, dose interruption and dose reduction are central to support. Growth-factor use is an individual clinical decision, not an automatic routine injection on this card.
- Discuss hot flushes, joint symptoms, vaginal symptoms, sleep and bone health according to the actual endocrine partner. Support can be adjusted without silently changing either cancer medicine.
- Report mouth soreness, troublesome nausea/diarrhoea or fatigue so supportive treatment and hydration can be reviewed. New breathing symptoms must not be dismissed as expected fatigue.
- Routine IV-chemotherapy antiemetic, steroid or G-CSF orders are not populated. Review persistent nausea and prescribe individual symptom treatment if needed.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00414 v5a · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-20 · SACTCheck v0.84.9 · Regimen-level drafts; prescriber and oncology-pharmacy verification required